# Neratinib · extended adjuvant treatment

Neratinib is taken once daily with food for one year. The usual starting dose is 240 mg; the team may interrupt or reduce it for toxicity. Treatment starts less than one year after completing adjuvant trastuzumab.

Oral HER-family kinase inhibitor · selected medicine-specific effects. Full details and sources are in the complete reference.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

### 1 Bowel / diarrhoea

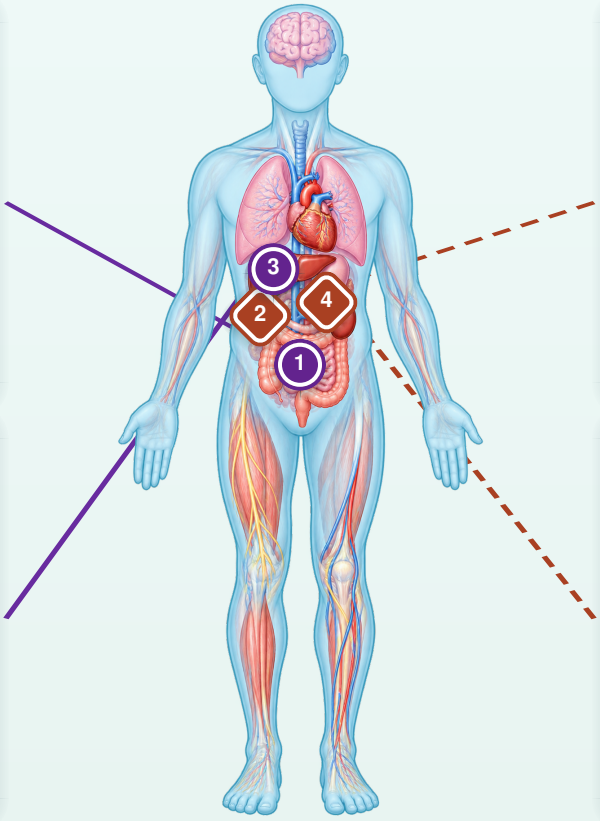
**Neratinib**

Start the prevention plan; report diarrhoea early.

### 2 Hydration / kidneys

**Neratinib-related fluid loss**

Poor intake, dizziness or reduced urine needs prompt advice.




### 3 Liver

**Neratinib**

Report jaundice, dark urine or upper abdominal pain.

### 4 Nausea / appetite

**Neratinib**

Call if nausea or vomiting prevents food, fluids or tablets.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report diarrhoea early. Poor intake, dizziness, reduced urine, blood in stool, fever, severe pain or jaundice needs prompt clinical advice. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

# Know the next step.

## Taking treatment

Neratinib is taken once daily with food for one year. The usual starting dose is 240 mg; the team may interrupt or reduce it for toxicity. Treatment starts less than one year after completing adjuvant trastuzumab.

Take tablets whole with food, preferably in the morning. Do not crush or dissolve them. Do not replace a missed dose. Start the prescribed loperamide prevention plan with the first neratinib dose.

## Checks and follow-up

full blood count (FBC) monthly for the first six months, then every three months. Liver tests at week 1, monthly for the first three months, then every six weeks; test earlier for liver symptoms or significant diarrhoea. Confirm normal baseline left ventricular ejection fraction (heart pumping measurement) (LVEF) eligibility; cardiac/LVEF assessment is clinically indicated. Review bowel symptoms, hydration, weight and interacting medicines. Continue any follow-up already required after prior trastuzumab; neratinib does not reset that plan.

The whole-treatment map shows the checks for this exact treatment setting.

## Support between reviews

### Diarrhoea prevention

Have the prescribed loperamide plan ready before the first tablet. Call early if diarrhoea starts or worsens.

### Hydration

Follow the fluid plan and report dizziness, poor intake or reduced urine.

### Liver checks

Keep the week-1 and subsequent liver tests; report yellow skin, dark urine or severe nausea.

### Other medicines

Check acid-suppressing medicines and all new prescriptions with the pharmacist.

**Oncology daytime:** Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital's urgent-care plan. Emergency: 112 / 999.

### Your complete treatment reference

https://sactcheck.com/patient/00720/

Public information only. No personal information is encoded.